MIND AI × ONTOPOLOGY

AI TCM Consultation and Prescribing

Complete output and usage plan

Mind AI × Ontypology (Space Type × Time Type)

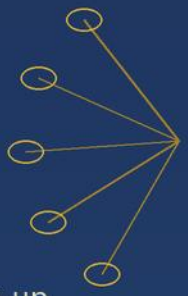


Look, listen, ask, feel

Pattern differentiation and treatment

Sovereign, minister, Assistant, courier





Four diagnostics × differentiation × prescribing × follow-up

Every consultation and prescription must have grounds

Formula and dosage for method illustration only, not a medical recommendation

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Before we begin: three conventions

This is a "how to use it" book. Its subject is AI TCM consultation and prescribing: starting from the four diagnostics of inspection, auscultation-olfaction, inquiry and palpation, moving through differentiation, pathomechanism, treatment method and prescribing, and on to follow-up adjustment - the whole road must be walked accurately. What this book gives you is neither a TCM textbook nor a system manual, but a complete plan for using the method accurately: every consultation and prescription produces an explicit "Type", and every Type passes its criteria and can be verified by evidence.

The theoretical foundation is Ontypology - the Space Type and the Time Type; the underlying platform is Mind AI. Before putting pen to paper, establish three conventions that apply throughout the book.

> **Convention one: "xíng" is Type**
>
> The English for "xíng" is always Type, never Form. Exam Type is ExamType, Syndrome Type is SyndromeType, Formula Type is FormulaType - terminology is consistent throughout the book.

> **Convention two: theory is used, not taught**
>
> This book does not restate the principles of Ontypology but takes the tools and uses them directly: the three brackets [ ] ( ) { }, the intersection criterion ∩(T), the identity anchor id(C), the three remove levels, the four-step framework, the three-layer judgment, and the Time Type's move board. For where these tools come from, return to the original texts on the Space Type and the Time Type; this book is only responsible for applying them to consultation and prescribing.

> **Convention three: the platform is cited, not described**
>
> Mind AI is the underlying platform; this book only cites its division-of-labour principle - LLM handles language, Mind Model handles intelligence - and does not restate the platform itself. What you need to know is this: every output of consultation and prescribing runs on this platform's reasoning, memory and learning capacity.

Important statement: all formula and herb content appearing in this book (including herbs, doses and modifications) serves only to demonstrate the method and does not constitute medical advice; for clinical care please have a qualified TCM physician diagnose and prescribe.

Finally, gather the whole book with one task map: each of the six chapters handles one stretch of the road, with output Types and tools accompanying it all the way.

Table 0-1 Task map: six stretches of road and their outputs

Chapter	Topic	What you must learn	Core tools
Chapter 1 (orientation)	What AI TCM Consultation and Prescribing Is	Understand the reason-able replica and where the two Types land	Type plus Number, the move board

Chapter	Topic	What you must learn	Core tools
Chapter 2 (output)	the six output Types	Structure every output of consultation and prescribing	Three brackets and the three remove levels
Chapter 3 (accuracy)	four gates of accuracy	Can state "why it is accurate"	∩(T), id(C), evidence chain
Chapter 4 (operation)	The six-step flow	Follow the steps along the whole road	The four-step framework, the three-layer judgment
Chapter 5 (drill)	Prompts and the case	S1 to S7, follow and use	seven prompts
Chapter 6 (advanced)	Misuse, checklists, maturity	Avoid the traps, assess which level you are at	The ten-question checklist, L1-L4

How to read this book: three routes

The language of Chinese medicine and the language of Type plus Number are two ways of saying the same thing. "Heart-spleen dual deficiency" is the conclusion of medical reasoning; the Syndrome Type is the structured spelling of that same conclusion. Translating between the two languages is the core skill this book teaches you.

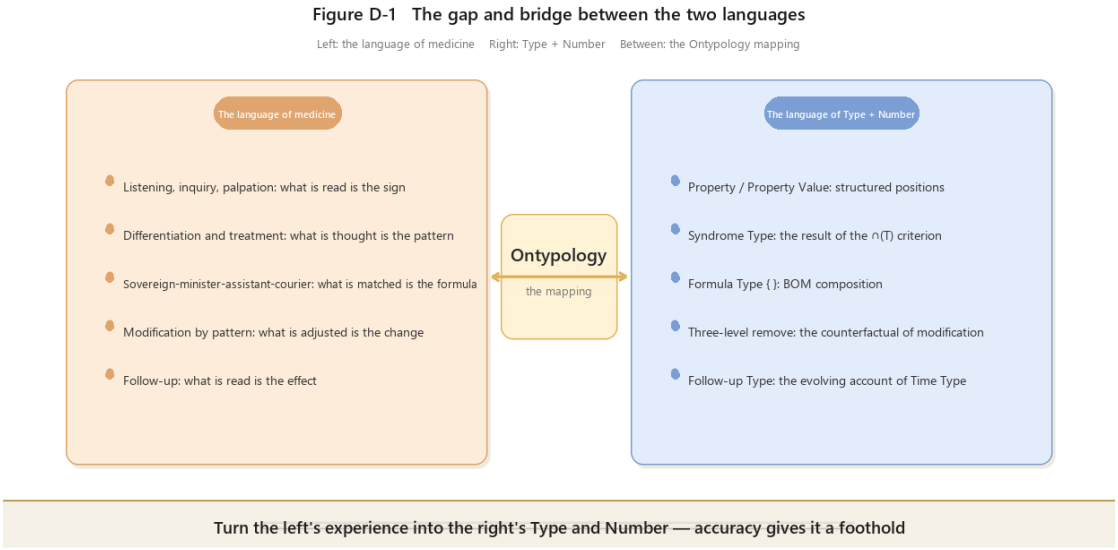


Figure D-1 The gap and the bridge between the two languages

Table D-1 Comparison of the two languages

The language of medical reasoning	The language of Type plus Number	Key points of translation
Inspection, auscultation-olfaction, inquiry and palpation	Exam Type	Findings into fields: only with fields can they be looked up
Differentiation (heart-spleen dual deficiency)	Syndrome Type	The feature set F(T) passes the ∩(T) criterion
Aetiology and pathomechanism	Mechanism Type	The causal chain of cause → mechanism → pattern
Therapeutic principle and treatment method	Treatment Type	One pathomechanism, one method, corresponding link by link
Prescription (sovereign-minister-assistant-courier)	Formula Type	BOM composition: {sovereign, minister, assistant, courier}
Follow-up adjustment	Follow-up Type	Therapeutic-effect comparison + version record + evolution

This book has three routes; pick one to suit your needs, there is no need to chew through it from cover to cover.

Table D-2 Three ways of reading

Reading path	who it suits	Route
Use directly	For those who want to prescribe right away	Copy the S1-S7 prompts of Chapter 5 and compare the outputs against the case examples

Reading path	who it suits	Route
For understanding	For those who want to explain the method clearly	Output Types in Chapter 2 → the four layers of accuracy in Chapter 3 → the drill in Chapter 5
For orientation	For those who want the team to keep getting stronger	Chapter 1 → the six steps of Chapter 4 → maturity L1-L4 in Chapter 6

The whole book demonstrates with one running case: Ms. Lin, thirty-two, insomnia for about two months. From four-diagnostic collection all the way to follow-up adjustment, every chapter returns to her. You may first turn to Chapter 5 for the complete walkthrough and then come back for the theory, or read straight through and watch the same case dissected layer by layer.

Chapter One Understanding AI TCM Consultation and Prescribing



Get the object clear first. This chapter answers four things: what the AI TCM brain actually is, where Type and Number land in Chinese medicine, how the two Types are used in consultation and prescribing, and where the word "accuracy" comes from. After this chapter you should be able to explain this method to a colleague in one sentence.

1.1 A Reason-able Consultation-and-Prescription Replica

What runs through an experienced physician's mind during a consultation? One look at the complexion, one touch on the pulse, a few sentences of the chief complaint, and the pattern type surfaces, with the formula taking shape alongside it. This line of thinking lives inside his head - implicit and hard to pass on. AI TCM consultation and prescribing has exactly one job: to turn that line of thinking into a reason-able replica - not only giving the answer but preserving the path to it.

Figure 1-1 Turn the physician's judgment into a reasoning copy

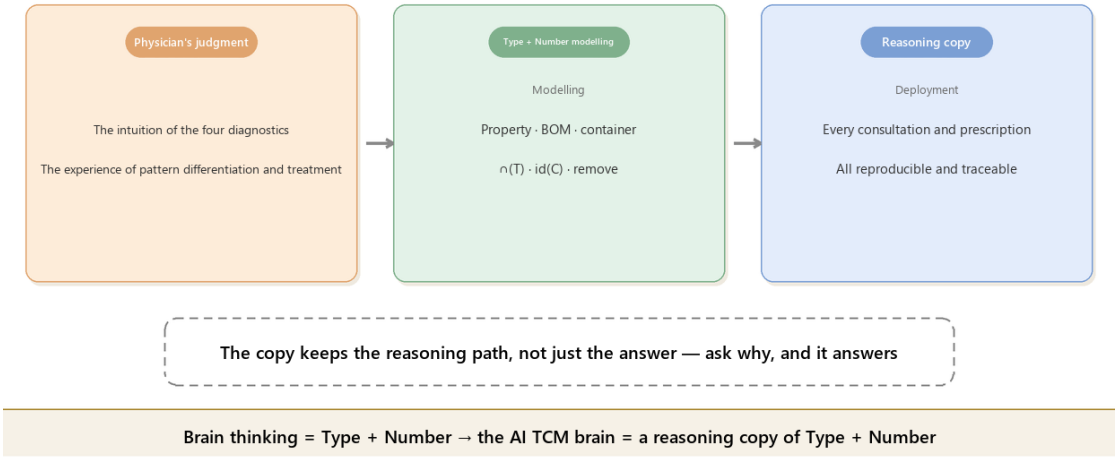


Figure 1-1 Turning the physician's judgment into a reason-able replica

The key to the replica is that it is reason-able. Ask it why it prescribed Guipi Decoction and it can

answer: because the pattern type is heart-spleen dual deficiency; because a thin-weak pulse with palpitations and forgetfulness passes this pattern type's criteria; because the treatment method is to tonify and benefit the heart and spleen and to nourish the blood and calm the spirit; therefore the formula comes out of the method. Behind the answer there is a path you can question, and that path is paved with Type and Number.

1.2 Type and Number Land in Chinese Medicine

The brain thinks in Type plus Number. Number describes quantity - dose, pulse rate, days of illness; Type describes structure - what a pattern type looks like, what a formula is composed of, how the pathomechanism links up. Chinese medicine lacks neither: what it lacks is a language that writes "Type" clearly.

Figure 1-2 Where Type and Number each land in Chinese medicine

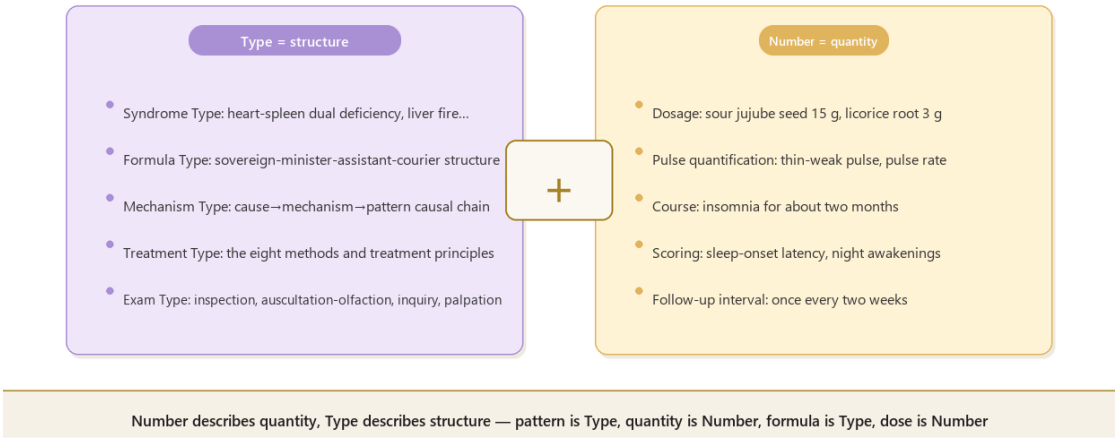


Figure 1-2 Where Type and Number each land in Chinese medicine

Traditional Chinese medicine emphasises "image" and "intuitive realisation", with little structuring - so from the same four-diagnostic information two physicians may write two different formulas, and neither can say exactly which link differs. Once Types are erected, the difference is forced onto the table: is it that F(T) was collected differently? Is it that the pathomechanism chain broke a link? Or is it that the treatment method took another route? Only when you can tell, can you speak of accuracy.

Table 1-1 Where Type plus Number land in Chinese medicine

Layer	Type (structure)	Number (magnitude)
Four diagnostics	Exam Type: four fields for inspection, auscultation-olfaction, inquiry and palpation	Tongue colour, pulse rate, days of illness
Differentiation	Syndrome Type: F(T) and ∩(T)	Symptom severity scoring
pathogenesis	Mechanism Type: the cause→mechanism→pattern chain	Stage of the illness
Treatment method	Treatment Type: therapeutic principle plus classification among the eight methods	Course length
Formula	Formula Type: the sovereign-minister-assistant-courier BOM	Herb and dose
Follow-up	Follow-up Type: version record	Therapeutic-effect difference

1.3 How the Two Types Are Used in Consultation and Prescribing

The Space Type and the Time Type each handle half of consultation and prescribing. The Space Type handles "seeing clearly": it erects the structure of person, pattern and formula - properties are the X axis, composition the Y axis, the container the Z axis, and the three axes pin an object in the coordinate system. The Time Type handles "carrying through": diagnosis, treatment and adjustment are a series of actions, and each action is a move - Time Type = verb = function = move.

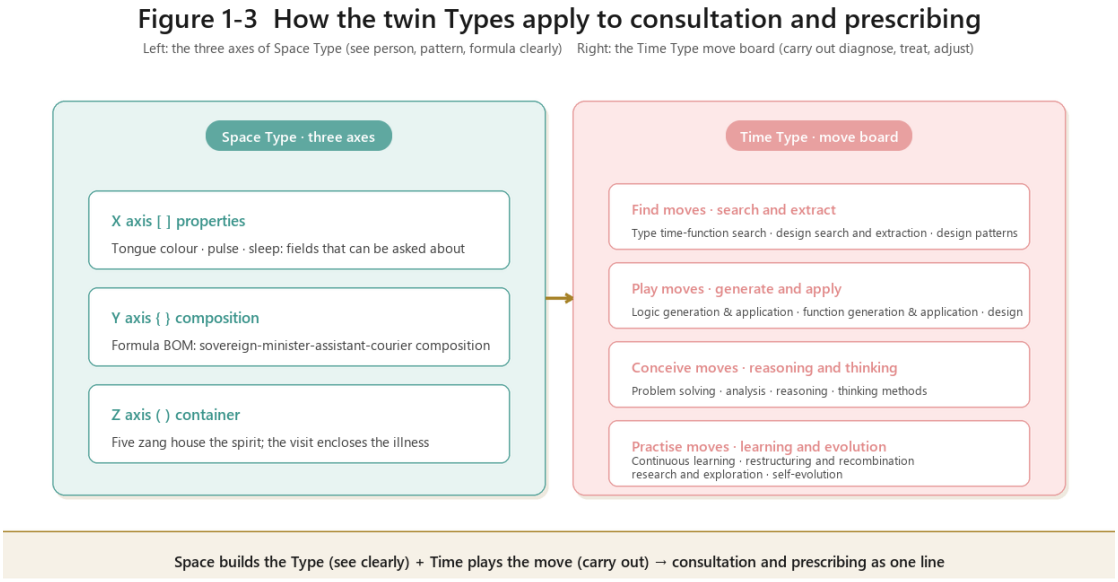


Figure 1-3 How the two Types are used in consultation and prescribing

Table 1-2 The Space Type's three axes × Chinese medicine

axis	Bracket	Where Chinese medicine lands
X axis · properties	[ ]	Tongue colour, pulse, sleep, bowels and urine ... the fields that can be asked about; property values are the options within a field
Y axis · composition	{ }	Formula BOM: which herbs make up sovereign, minister, assistant and courier; the three remove levels correspond to the scale of modification
Z axis · container	( )	The body is the container (the five zang organs house the spirit; qi, blood and fluids); the visit encloses the illness; id(C) anchors identity

The Time Type's move board has four groups and fourteen moves. This book lists only the headings and takes them directly; for the definitions and derivations of the moves, return to the original text on the Time Type.

Table 1-3 The Time Type's move board × Chinese medicine (four groups)

Set	Move	How to use consultation and prescribing
Find moves	Type time-function search · design search and extraction · design patterns	Find similar cases, extract pattern-type features, apply established formula combination patterns
Make the move	Logic generation and application · function generation and application · design	Pathomechanism inference, treatment-method generation, formula composition and modification (prescribing is design)
Devise moves	Problem solving · analysis · reasoning · thinking methods	Pattern differentiation, causal reasoning, reverse testing of the pattern from the formula, differential diagnosis

Set	Move	How to use consultation and prescribing
Practise the moves	Continuous learning · restructuring and recombination · research and exploration · self-evolution	Case archiving, adjustment restructuring, exploring new formulas, ever more accurate

Join the two halves: spatial Type-building gives "person, pattern, formula" structure, and temporal move-making gives "diagnose, treat, adjust" action. The six output Types (Chapter 2) are the crystallisation of space, the six steps (Chapter 4) are the routines of time, and S1 to S7 (Chapter 5) are the concrete hand movements of each move.

1.4 Division of Labour with Mind AI as the Underlying Platform

This method runs on Mind AI. The division-of-labour principle in one sentence: LLM handles language, Mind Model handles intelligence. The LLM is responsible for understanding the chief complaint, writing the four-diagnostic record and recalling formula literature; the Mind Model is responsible for unifying knowledge, deep reasoning, long-term memory and continuous learning. The deduction of pattern differentiation and treatment is borne by the Mind Model, and the entry and exit of language by the LLM.

Figure 1-4 Division of labour on the Mind AI platform

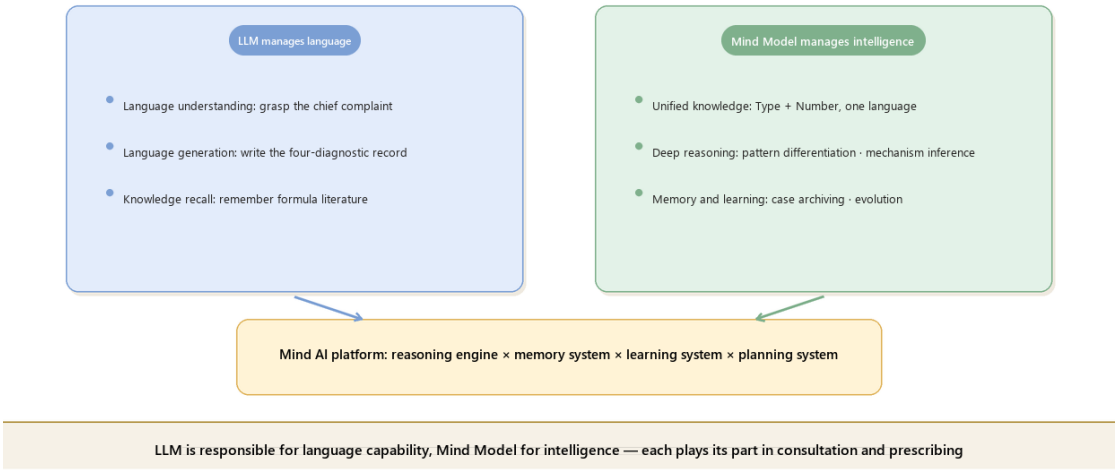


Figure 1-4 Division of labour with Mind AI as the underlying platform

The division of labour lands on every station of consultation and prescribing: the S1 four-diagnostic record is written by the LLM, the pattern differentiation and pathomechanism inference of S2 and S3 are reasoned by the Mind Model, literature lookup at S5 is joint work, and the case archiving and experience write-back of S7 are closed by the Mind Model's memory and learning capacity. Language input and output are never absent, and intelligent reasoning is never outsourced - that is the meaning of the underlying platform.

1.5 Where Accuracy Comes From

"Can an AI-written formula be trusted?" This question demands a structural answer, not a feeling. This book's answer is the four-layer accuracy guarantee, unfolded layer by layer in Chapter 3; here is one sentence previewing each: differentiation has criteria (only what passes ∩(T) enters the pattern type), inference has a skeleton (the pathomechanism chain links up), composition has validation (sovereign-minister-assistant-

courier plus combination contraindications), and the whole process has evidence (every step points back to the four diagnostics). Only when all four layers pass may a formula leave.

The other half of accuracy comes from the temporal dimension: follow-up turns every therapeutic effect into experience for the next visit, the case file grows thicker and the criteria sharper. The four spatial layers govern "is this formula accurate"; temporal evolution governs "is the next formula more accurate".

Chapter focus

The AI TCM brain is a reason-able replica of consultation-and-prescription thinking: behind every answer there is a path you can question.

Type describes structure, Number describes magnitude; erect the Type and accuracy finally has a consistent language.

The Space Type's three axes see "person, pattern, formula" clearly; the Time Type's move board carries "diagnose, treat, adjust" through.

Mind AI division of labour: LLM handles language, Mind Model handles intelligence; accuracy comes from four layers of guarantee plus temporal evolution.

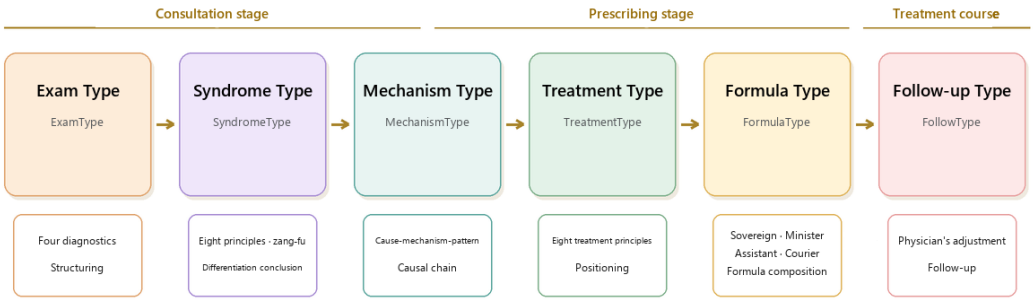
Chapter Two Six Output Types: What Consultation and Prescribing Must Produce



Consultation and prescribing is not a one-off conversation but a relay of six outputs: Exam Type, Syndrome Type, Mechanism Type, Treatment Type, Formula Type and Follow-up Type. Each is a Type, each with explicit fields and criteria. Once the Type is fixed, outputs become accumulable, comparable and acceptable.

Figure 2-1 The six output Types of consultation and prescribing

Every station has a clear input and output; every station checkable, every link askable



Four diagnostics → pattern → mechanism → treatment → formula → follow-up: each step is a Type

Figure 2-1 The six output Types of consultation and prescribing

Memorise the summary table first; the six sections that follow take it apart. Each section answers the same three things: what this Type's fields are (the X axis of properties), what it is composed of (the Y axis of composition), and which container it lives in (the Z axis of the container).

Table 2-1 Master table of the six output Types

Output Type	English name	One sentence	Criteria
Exam Type	ExamType	A structured record of inspection, auscultation-olfaction, inquiry and palpation	Findings complete, no invented filling

Output Type	English name	One sentence	Criteria
Syndrome Type	SyndromeType	The conclusion of pattern differentiation	∩(T)
Mechanism Type	MechanismType	The causal chain of cause → mechanism → pattern	Every link has four-diagnostic evidence
Treatment Type	TreatmentType	The bridge from pattern to formula	One pathomechanism, one method, link by link
Formula Type	FormulaType	The composition of sovereign, minister, assistant and courier	Combination validation + the three remove levels
Follow-up Type	FollowType	The Time Type of a course of treatment	Before-and-after comparison plus version record

2.1 Exam Type: Structuring Inspection, Auscultation-Olfaction, Inquiry and Palpation

The Exam Type is the foundation of the whole road. It turns inspection, auscultation-olfaction, inquiry and palpation into four field groups, each with fillable properties and property values - a property is "a question that can be asked", and a property value is this visit's answer to it. Fix the fields and no information is lost; a missing field shows "not collected" instead of being silently skipped.

Table 2-2 Exam Type field table

Field group	Core properties	Property value examples (Ms. Lin)
Inspection	Complexion, body build, tongue colour, tongue coating, demeanour	Pale complexion; rather thin build; pale tongue; thin white coating; listless spirit
Auscultation-olfaction	Voice, breath, special odours	Faint voice; no special odour
Inquiry	Sleep, palpitations, appetite, bowels and urine, menstruation, emotions	Difficulty falling asleep with much dreaming and easy waking; palpitations and forgetfulness; poor appetite and dull intake; normal bowels and urine; scanty pale menses; much brooding
Palpation	Pulse, abdominal diagnosis, tenderness	Thin-weak pulse; abdomen soft without tenderness

In practice there are two iron rules. First, objective findings come first: tongue and pulse findings are mandatory; if absent, mark "not collected" and put them on the follow-up question list, never fill them in by guessing. Second, degree and duration must go into the property values: "insomnia for about two months" and "waking two to three times a night" are far more useful than "sleeps badly" - the feature set F(T) used in later differentiation grows out of exactly this.

2.2 Syndrome Type: The Conclusion of Pattern Differentiation

The Syndrome Type is the conclusion of pattern differentiation, written as a two-level structure like "insomnia · heart-spleen dual deficiency pattern": disease name (insomnia) + pattern type (heart-spleen dual deficiency). It is not a sentence but a membership judgment of a set - [insomnia] is an abstract set, and for a patient to enter it the feature set F(T) must pass this pattern type's intersection criterion ∩(T).

Figure 2-2 Three kinds of brackets, one insomnia

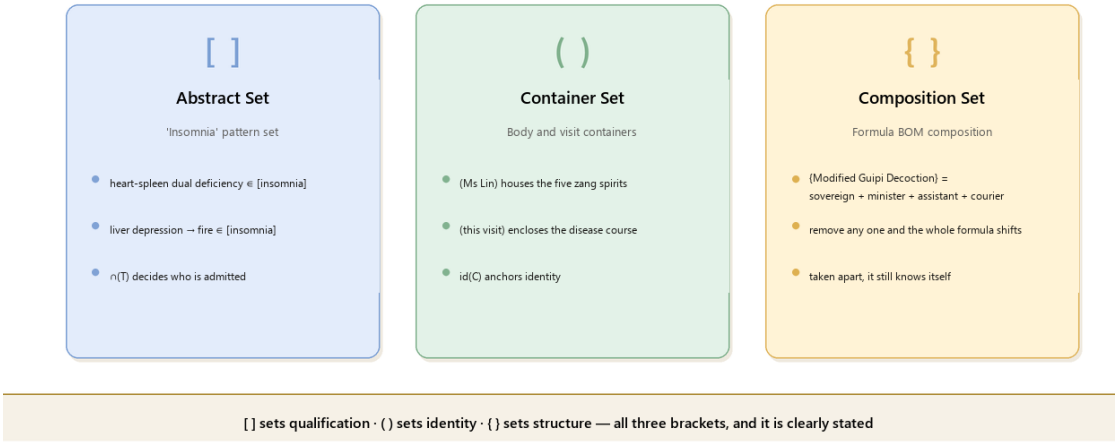


Figure 2-2 Three brackets, the same insomnia

Each of the three brackets has its duty at the point of differentiation: [ ] sets eligibility - does this presentation belong to insomnia, is it heart-spleen dual deficiency; ( ) sets identity - Ms. Lin is a container, inside which the five zang organs, spirit, qi and blood reside, (this visit) encloses this stretch of illness, and id(C) anchors the person and the illness; { } sets structure - what the formula is composed of, used in the next section and in 2.5.

Table 2-3 Syndrome Type: localisation and criteria

Steps	Content	Ms. Lin's judgment
Locating the disease name	Insomnia: chief complaint of inability to obtain normal sleep	Chief complaint of insomnia for about two months → disease name insomnia
Locating by the eight principles	Yin and yang, exterior and interior, cold and heat, deficiency and excess	Interior, deficiency, no marked cold signs → interior deficiency pattern
Zang-fu localisation	Which of heart, spleen, liver and gallbladder is involved	Palpitations and forgetfulness + poor appetite and dull intake → heart and spleen
F(T) comparison	The feature set compared one by one against candidate pattern types	Pale complexion, pale tongue, thin-weak pulse, scanty menses
Exclusion differential	Why a similar pattern type is not chosen	Not liver depression transforming into fire (no irritability, bitter taste or wiry pulse)

2.3 Mechanism Type: The Middle Segment of the Causal Chain

The Syndrome Type answers "what it is", the Mechanism Type answers "why". It strings aetiology, pathomechanism and pattern manifestations into a causal chain: excessive brooding stealthily consumes heart blood, the spleen fails to transport, qi and blood have no source, the spirit is deprived of nourishment, and so there is insomnia. This chain is the middle segment of the deduction skeleton - it follows the four-diagnostic evidence and precedes the treatment method, formula and herbs (detailed in Chapter 3).

The Mechanism Type has two field rules. First, every link must point back to evidence in the four diagnostics: to say "the spleen fails to transport" there must be findings such as poor appetite, dull intake or fatigue holding it up; a link that cannot point back is speculation and must be cut. Second, the chain is ordered: scramble the causal order and the treatment method will seize the wrong focus - whether to tonify the blood

first or strengthen the spleen first depends on which link is the main mechanism.

2.4 Treatment Type: The Bridge from Pattern to Formula

The Treatment Type is the bridge between pathomechanism and formula: the pathomechanism says "heart blood insufficient, spleen transport not robust, spirit deprived of nourishment", so the treatment method is set as "tonify and benefit the heart and spleen, nourish the blood and calm the spirit" - one pathomechanism, one method, corresponding link by link, no more and no less. The therapeutic principle is the general direction (tonify what is deficient), the treatment method is the concrete landing point (tonify heart and spleen, nourish the heart and calm the spirit), and with the classification among the eight methods added (this case belongs to the tonifying method), the three columns are complete and the Treatment Type is whole.

The Treatment Type has exactly one criterion: once the formula is written, every herb must point back to the treatment method, and every clause of the treatment method must point back to the pathomechanism. If the bridge is built with shoddy work, the formula becomes a heap of herbs - which is precisely the commonest of the five kinds of misuse in Chapter 6.

2.5 Formula Type: The Composition of Sovereign, Minister, Assistant and Courier

The Formula Type is this book's most important output; it is simply a BOM (a composition set { }): the base formula is the root, the four grades of sovereign, minister, assistant and courier are the trunk, and herbs and doses are the leaves. The sovereign herb targets the main pattern and main mechanism; the minister assists the sovereign; the assistant attends to concurrent patterns or guards against side effects; the courier harmonises the herbs and guides the formula to the channel. Every herb has a name, a dose and a task - missing any one of the three and the BOM is incomplete.

Figure 2-3 Formula Type: sovereign-minister-assistant-courier is a BOM

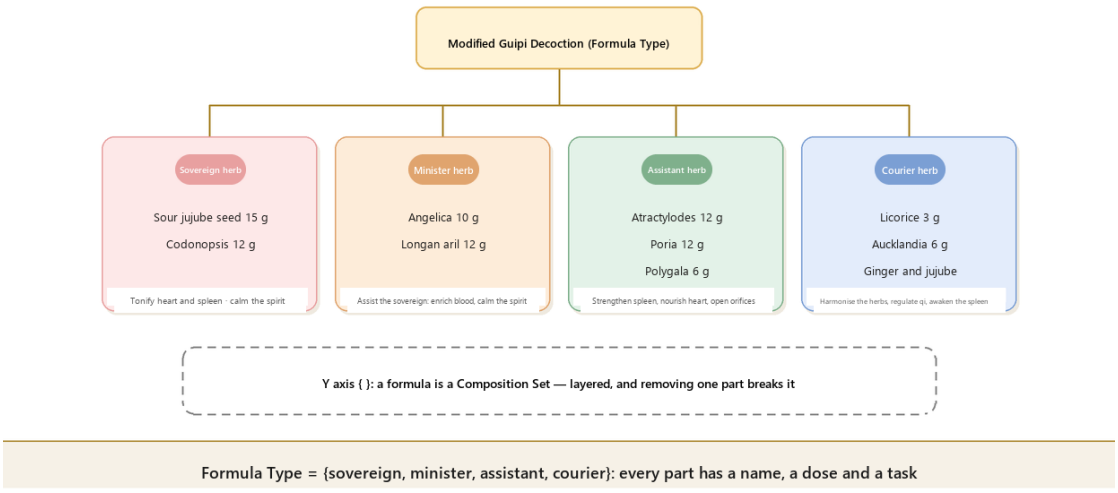


Figure 2-3 Formula Type: sovereign-minister-assistant-courier is a BOM

Table 2-4 Formula Type BOM: Guipi Decoction with modifications (for demonstration only)

level	Herbs and doses	Task
Sovereign	Astragalus root 15 g, codonopsis root 12 g, sour jujube seed 15 g	Tonify and benefit the heart and spleen, nourish the heart and calm the spirit, aimed straight at the main mechanism
Minister	Angelica root 10 g, longan aril 12 g, Poria with hostwood 12 g	Nourish the blood and calm the spirit, assisting the sovereign
Assistant	Atractylodes rhizome 12 g, polygala root 6 g	Strengthen the spleen and assist transport, calm the spirit and open the orifices, and attend to concurrent patterns
Courier	Aucklandia root 6 g (added near the end), licorice root 3 g, fresh ginger 3 slices, jujube 5 pieces	Regulate qi and awaken the spleen so that tonification does not stagnate, and harmonise the herbs

Modification is the dynamic field of the Formula Type, and its scale is decided by the three remove levels: single herb, herb pair (corner), formula root (base formula). Before modifying, ask the counterfactual question - "take it away; does it still recognise itself?" Remove a single herb and the question is whether a task goes unfulfilled; remove an herb pair and the question is whether a pairing fails; remove the formula root and the question is whether the whole formula's skeleton still stands. Only when all three questions pass may you move at that level.

Figure 2-4 Three-level remove: before modifying, ask whether it stands alone

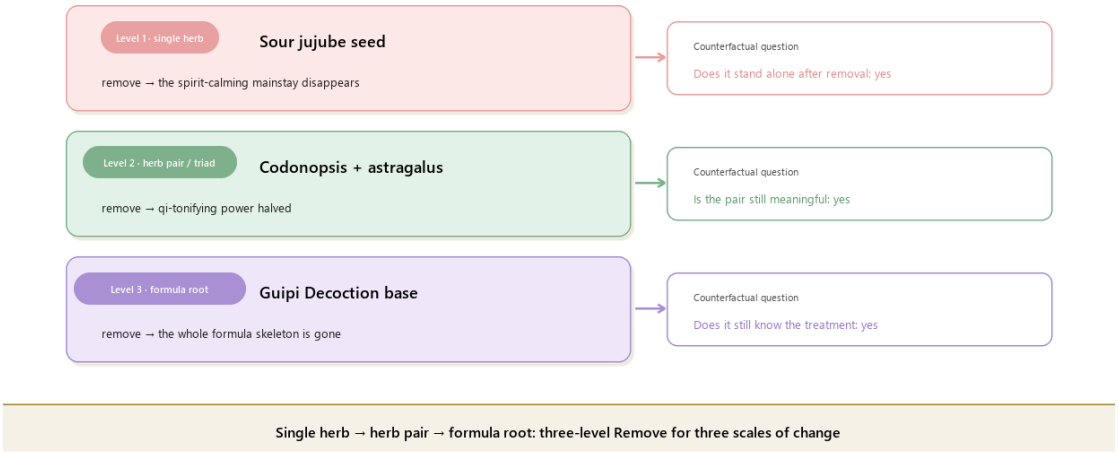


Figure 2-4 The three remove levels: before modifying, ask whether it still recognises itself when taken apart

Table 2-5 The three remove levels × scale of modification

Level	Object	The counterfactual question	Modification examples (for demonstration only)
Level one	Single herb	Take it away: does the task go unfulfilled	Poor sleep with much dreaming → add Biota seed 10 g to strengthen the spirit-calming task
Level two	Herb pair / corner	Take them away: does the pairing fail	Codonopsis with astragalus is a qi-tonifying pair; do not lightly split it unless the formula is changed

Level	Object	The counterfactual question	Modification examples (for demonstration only)
Level three	Formula root / base formula	Take the skeleton away: does the treatment method still stand	Only if the pattern type changes (e.g. to liver depression transforming into fire) is the base formula changed

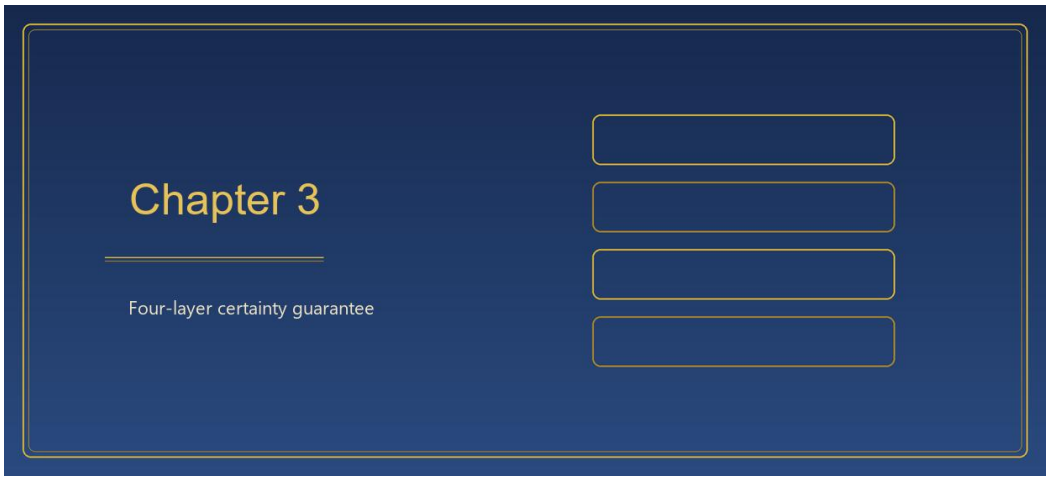
2.6 Follow-up Type: The Time Type of a Course of Treatment

The Follow-up Type is the Time Type of a course of treatment: it stretches "diagnose, treat, adjust" into a timeline. There are four groups of fields - therapeutic-effect comparison (the difference between the before and after four diagnostics), pattern-type change (pattern kept or pattern changed), adjustment decision (keep, modify or change the formula, with reasons), and evolution record (what was learned this time, written back into the case file). Follow-up is not a fresh consultation but a comparison against the previous case file.

id(C) matters especially here: the container's identity must be anchored, so that "this stretch of Ms. Lin's illness" can be tracked, compared and accumulated. Every follow-up is a new version of the case file, and the old version is never overwritten - this is exactly where the Time Type's two moves, continuous learning and self-evolution, land.

> **Chapter focus**
>
> Six output Types relayed: four diagnostics → pattern type → pathomechanism → treatment method → formula → follow-up; each Type has its fields and criteria.
>
> Division of labour among the three brackets: [ ] sets eligibility, ( ) sets identity, { } sets structure; the pattern type is a membership judgment of the set.
>
> The Formula Type is a BOM: sovereign, minister, assistant and courier are clearly graded, and every herb has a name, a dose and a task.
>
> The scale of modification is decided by the three remove levels; the Follow-up Type anchors the course with id(C) so experience can accumulate.

Chapter Three Four-Layer Accuracy Assurance



Accuracy cannot rest on feeling. This chapter breaks "why this formula can be trusted" into four layers: differentiation has criteria, inference has a skeleton, composition has validation, and the whole process has evidence. The four layers are four gates, each with its own question and its own return handling.

Figure 3-1 The four-layer accuracy guarantee: from criteria to evidence

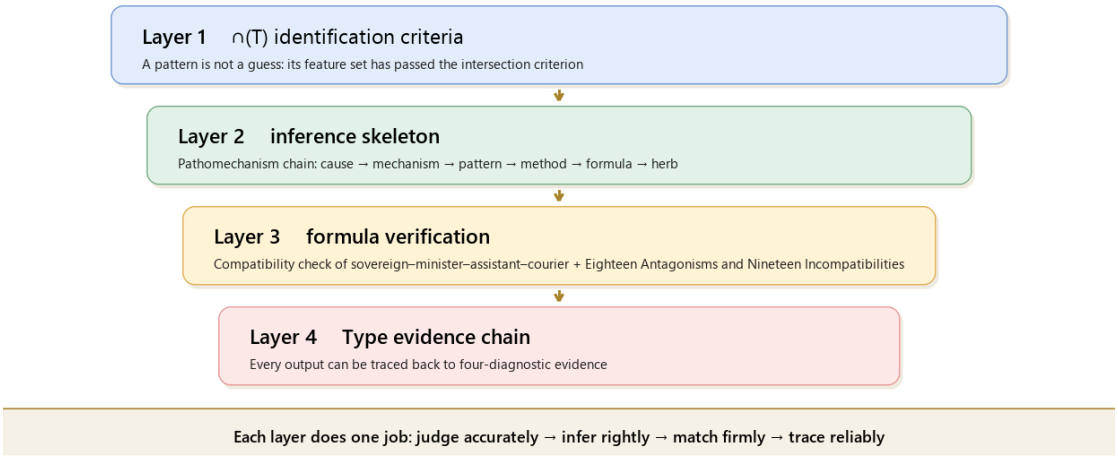


Figure 3-1 The four-layer accuracy guarantee: from criteria to evidence

Each of the four layers has its own return handling: if ∩(T) fails, return to supplement the four diagnostics; if the pathomechanism chain breaks a link, return and re-derive; if combination is doubtful, return and recompose; if the evidence chain breaks, return and complete it. Only with explicit return paths does the pipeline dare release work to the next gate - S1 to S7 in Chapter 5 put each of these return paths into the prompts.

3.1 Layer One: the ∩(T) Pattern-Differentiation Criterion

The first layer governs differentiation. A pattern type T is decided by two things: the feature set F(T) and the intersection criterion ∩(T). The patient's presentations make up a feature set, and candidate pattern types each have their own criteria - the two are intersected, and only what passes is admitted. This is not a

lookup comparing a single symptom but a set judgment over the whole group of features, which is why the same chief complaint (insomnia) is triaged into four different pattern types and four different formulas.

Figure 3-2 ∩(T) differentiation criterion: the funnel separating four insomnia pattern types

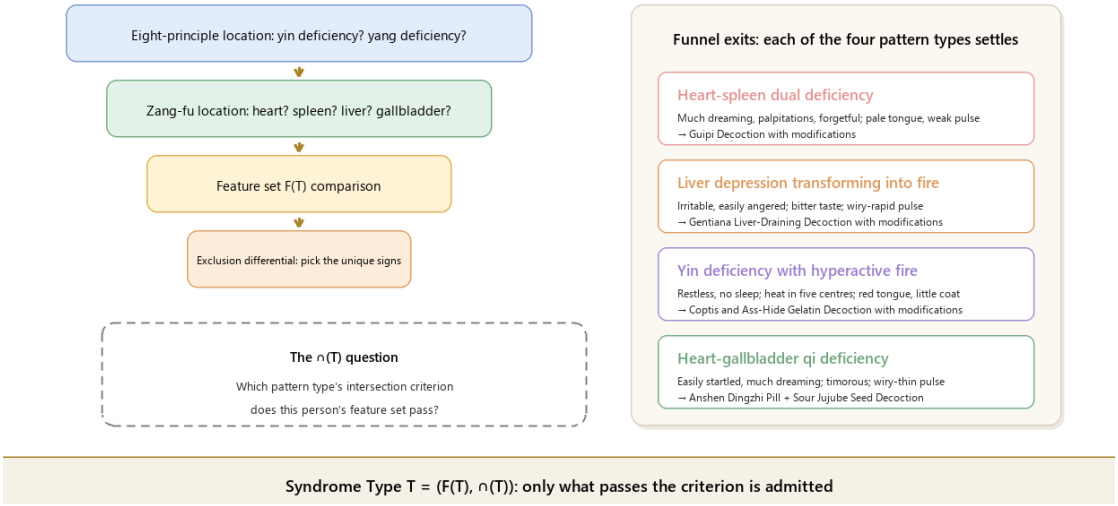


Figure 3-2 The ∩(T) differentiation criterion: the funnel separating four insomnia pattern types

Table 3-2 ∩(T) differentiation: four insomnia pattern types (formulas for demonstration only)

Pattern type	F(T) key features	∩(T) unique signs	Base formula
Heart-spleen dual deficiency	Much dreaming and easy waking, palpitations and forgetfulness, poor appetite and fatigue	Pale complexion, thin-weak pulse	Guipi Decoction with modifications
Liver depression transforming into fire	Irritability and easy anger, bitter taste, constipation and dark urine	Wiry-rapid pulse, yellow coating	Gentiana Liver-Draining Decoction with modifications
Yin deficiency with hyperactive fire	Restlessness and insomnia, heat in the five centres, night sweats	Red tongue with little coating, thin-rapid pulse	Coptis and Ass-Hide Gelatin Decoction with modifications
Heart-gallbladder qi deficiency	Easily startled and much dreaming, prone to fright, short of breath	Wiry-thin pulse, easily startled by trifles	Anshen Dingzhi Pill combined with Sour Jujube Seed Decoction

The funnel's last gate is the exclusion differential: you must be able to write "why it is not another pattern type". Ms. Lin has no irritability or bitter taste and her pulse is not wiry-rapid, so liver depression transforming into fire is out; she has no heat in the five centres and her tongue is not red, so yin deficiency with hyperactive fire is out. Only when the exclusion can be written does it mean ∩(T) was really run once, rather than an answer recited.

3.2 Layer Two: the Deduction Skeleton, a Chain of Pathomechanism Inference

The second layer governs inference. From cause to herb is a six-link deduction chain: cause → mechanism → pattern → method → formula → herb. Each link is one deductive inference (a Time Type reasoning move), and each link needs four-diagnostic evidence or the support of the link before it. If any link on the chain breaks, everything after it is void - this is the structured version of "the formula comes from

the method, the method is established from the pattern".

Figure 3-3 The deduction skeleton: the pathomechanism chain, link locked into link

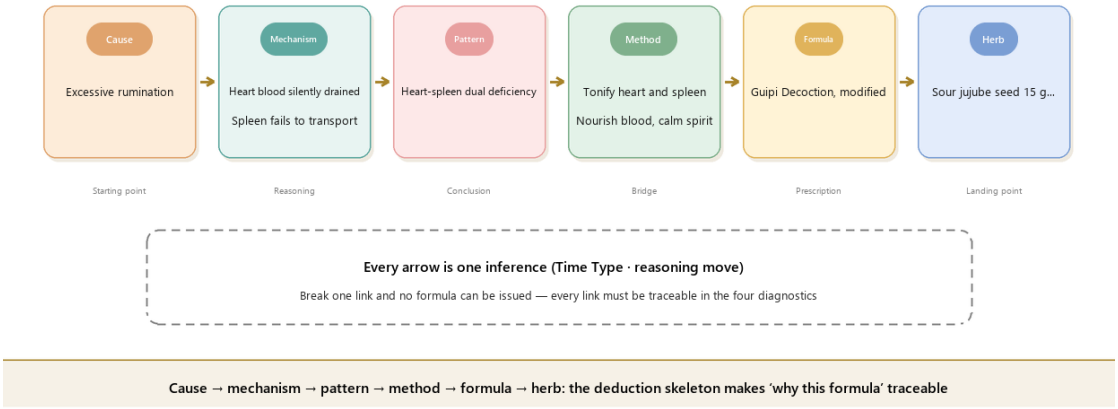


Figure 3-3 The deduction skeleton: the pathomechanism chain, link locked into link

Table 3-3 Pathomechanism inference chain fields (Ms. Lin example)

Link	Content	Supporting evidence
Cause	Excessive brooding	Inquiry: much brooding, clerical work
Mechanism	Stealthily consuming heart blood, the spleen failing to transport, qi and blood without a source	Palpitations and forgetfulness, poor appetite and dull intake, fatigue and weakness
Evidence	Heart-spleen dual deficiency (insomnia)	Pale complexion, pale tongue with thin white coating, thin-weak pulse
Method	Tonify and benefit the heart and spleen, nourish the blood and calm the spirit (the tonifying method)	Corresponding main mechanism: heart-spleen dual deficiency, spirit deprived of nourishment
Formula	Guipi Decoction with modifications	The base formula's indication matches the pattern type
Herb	Every herb of sovereign-minister-assistant-courier points back to the treatment method	See Table 2-4 and Table 5-4

3.3 Layer Three: Medical Validation of the Formula

The third layer governs composition. The deduction chain guarantees "the formula comes from the method", but even with the right method the formula can go crooked - hence validation. Validation has two gates: the structural gate and the safety gate. The structural gate checks whether sovereign, minister, assistant and courier are complete, whether each herb's task is clear, and whether any herb whose task cannot be stated has slipped in; the safety gate checks combination contraindications (the eighteen incompatibilities and nineteen mutual antagonisms), whether doses are within the customary range, and special-population warnings (pregnancy, menstruation, allergy history).

Validation is asked counterfactually - run the three remove levels again: take any single herb out of this formula, and which task goes unfulfilled? If you can answer, every herb is at its post; if you cannot, that herb is suspect. The handling of a failed validation is explicit: name the break point, return to S5 and recompose, and never release a formula carrying a doubt.

3.4 Layer Four: the Type Evidence Chain

The fourth layer governs verification. The Type evidence chain strings the six output Types into a retraceable road: the prescription points back to the treatment method, the treatment method to the pathomechanism, the pathomechanism to the pattern type, and the pattern type to the four diagnostics. When someone asks "on what grounds", every step can answer with the step before - like a medical record, and like the reasoning in a judgment.

Figure 3-4 Type-evidence chain: every step can be traced back

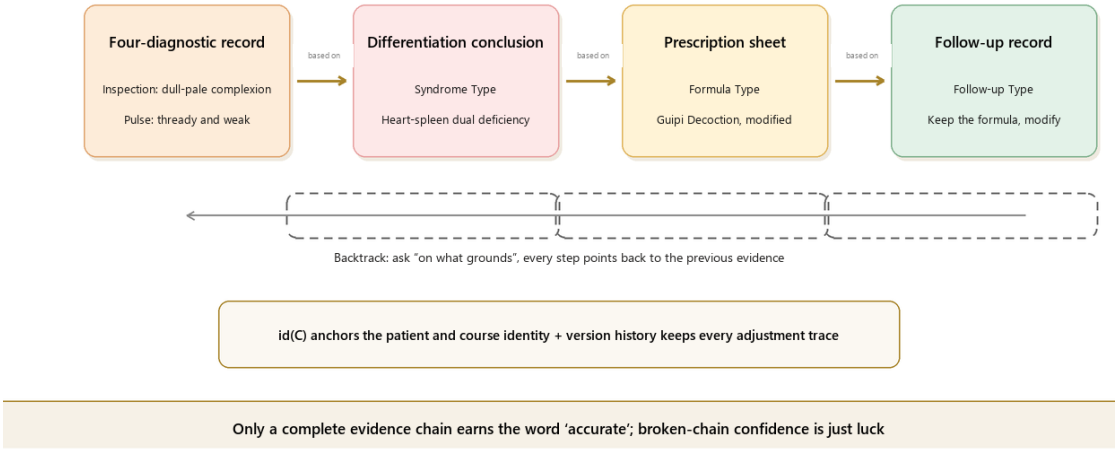


Figure 3-4 The Type evidence chain: every step can be traced back

Table 3-4 Type evidence chain fields

Output	What to store	Backtrack to
Exam Type	Original findings plus the not-collected list	The moment of consultation (dialogue record)
Syndrome Type	F(T) + ∩(T) comparison + exclusion differential	Exam Type
Mechanism Type	The causal chain link by link plus evidence references	Exam Type, Syndrome Type
Treatment Type	Therapeutic principle and treatment method plus the corresponding pathomechanism link	Mechanism Type
Formula Type	Sovereign-minister-assistant-courier plus reasons for modification plus the validation report	Treatment Type
Follow-up Type	Therapeutic-effect comparison + adjustment version	Previous case file (threaded by id(C))

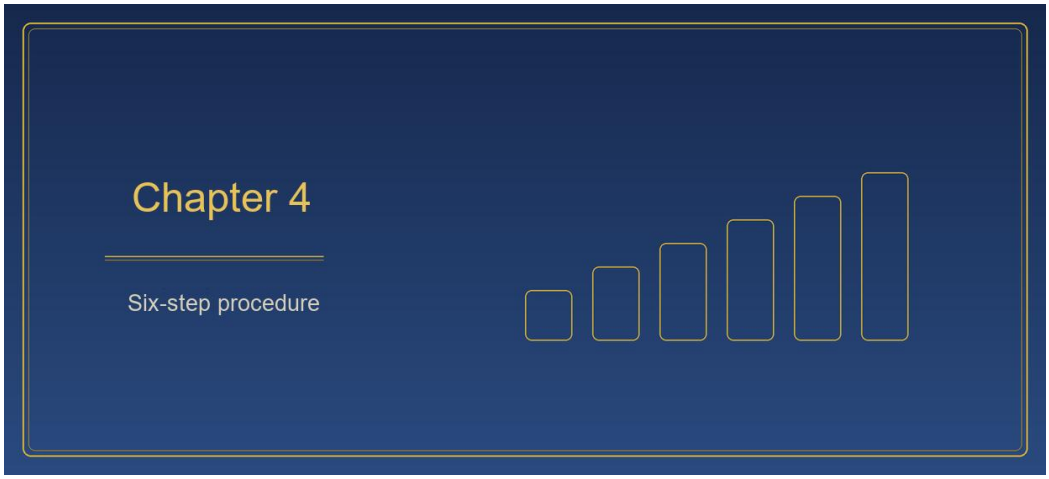
3.5 Cross-Acceptance of Space and Time

The four layers are the gatekeeping of the spatial view - this formula, accurate at this moment. Do not forget the temporal view: the Follow-up Type brings therapeutic-effect feedback, letting criteria be tested and corrected. Space × time cross-acceptance means taking the follow-up result back to compare with the original four layers: the heart-spleen dual deficiency judged then - did sleep improve after the formula? If it did, the four layers' judgment is reinforced; if not, the file must be reopened - was F(T) collecting the wrong signs, or did the pathomechanism chain seize the wrong main mechanism?

This reverse checking is the concrete action of the Time Type's "self-evolution": at every cross-acceptance the case file gains one more page of conclusions, and the ∩(T) criteria gain one more degree of refinement. The four layers govern the lower bound of accuracy; evolution governs its upper bound.

> **Chapter focus**
>
> Each of the four layers governs one thing: ∩(T) governs differentiation, the deduction chain governs inference, medical validation governs composition, and the Type evidence chain governs verification.
>
> Only when the exclusion differential can be written out has ∩(T) really been run; a pathomechanism chain with one broken link invalidates everything after it.
>
> Validation passes the structural gate and the safety gate; the remove counterfactual question is the fastest self-check.
>
> The four spatial layers govern this formula; temporal evolution governs the next one; cross-acceptance makes the criteria sharper with use.

Chapter Four The Six Steps: An Operating Procedure for Using the Method Accurately



The output Types are the "things"; this chapter is the "actions". The six steps line up the outputs and criteria of the previous two chapters into a road that can be walked repeatedly: clarify the situation, build Types along the axes, pick the expansion pattern, generate output, criteria acceptance, archive and evolve. Walk it once per visit and the case file grows one page thicker.

Figure 4-1 Six steps: turning a method into a path

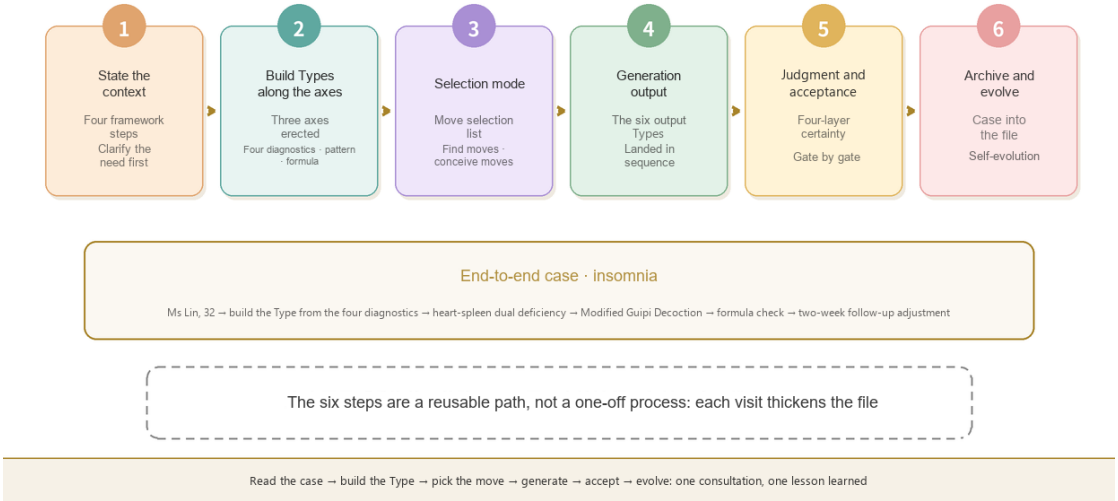


Figure 4-1 The six steps: walking the method into a road

4.1 Start with the Four-Step Framework to Clarify the Need

Before the six steps, run the four-step framework - this is the opening move of the Space Type, used to clarify "what this visit needs" instead of plunging into a pile of symptoms. The four steps in order are: fix the object (whose container is this, what is id(C)), fix the scope (what this visit will handle and what it will not), fix the goal (what the therapeutic-effect indicator is), and fix the constraints (constitution, menstrual period, drug allergies, life constraints).

Table 4-1 The four-step framework × consultation and prescribing

Steps	Questions to ask	Ms. Lin example
Fix the object	Whose container is it? What is id(C)?	Ms. Lin, 32, clerical work; this stretch of illness = insomnia for about two months
Fix the scope	What does this visit handle? What does it leave alone?	Handle the insomnia chief complaint; leave menstrual regulation alone for now and list it for later
Fix the goal	What is the therapeutic-effect indicator?	Sleep-onset latency shortened, night awakenings reduced, palpitations eased
Fix the constraints	Constitution, contraindications, life constraints?	Not pregnant; period approaching; avoid strong tea and coffee; high work stress

4.2 Walk the Six Steps Once

Once the framework is clear, the six steps make their moves in order. Note how they correspond to the output Types: the first three steps are preparation (situation, Types, moves), the fourth generates the six output Types, the fifth runs the four layers of accuracy, and the sixth settles the result into the Time Type's evolution.

Table 4-2 Summary of the six steps

Step	Action	Output	Corresponding chapter
1	Clarify the situation (four-step framework)	Situation statement: object, scope, goal, constraints	Section 4.1 of this chapter
2	Build Types along the axes (three axes)	Once the Exam Type fixes the fields, the skeleton of differentiation and formula is in place	Chapter 2 (output Types)
3	Pick the expansion pattern (the move board)	Move selection sheet: which moves this visit will use	Section 4.4 of this chapter
4	Generate the output	The six output Types land in order	Chapter 5 (drill)
5	Criteria acceptance (four layers)	A validation report; fail and the work is returned	Chapter 3 (accuracy)
6	Archive and evolve	Case into the file, experience written back, criteria refined	This chapter plus Chapter 6

4.3 Three-Layer Judgment: Body, Formula, Pattern

When you keep turning back to make judgments mid-operation, use the three-layer judgment of Type logic, asking three questions from outside inward: the container judgment - whose container does this question involve? The body is a container, inside which the five zang organs and the spirit reside; a visit is also a container, enclosing this stretch of illness, with identity anchored by id(C). The composition judgment - what is this formula composed of? The counterfactual questions of the three remove levels run here. The abstract judgment - does this presentation belong to [pattern type]? ∩(T) has the last word.

Figure 4-2 Three layers of judgment: body · formula · pattern

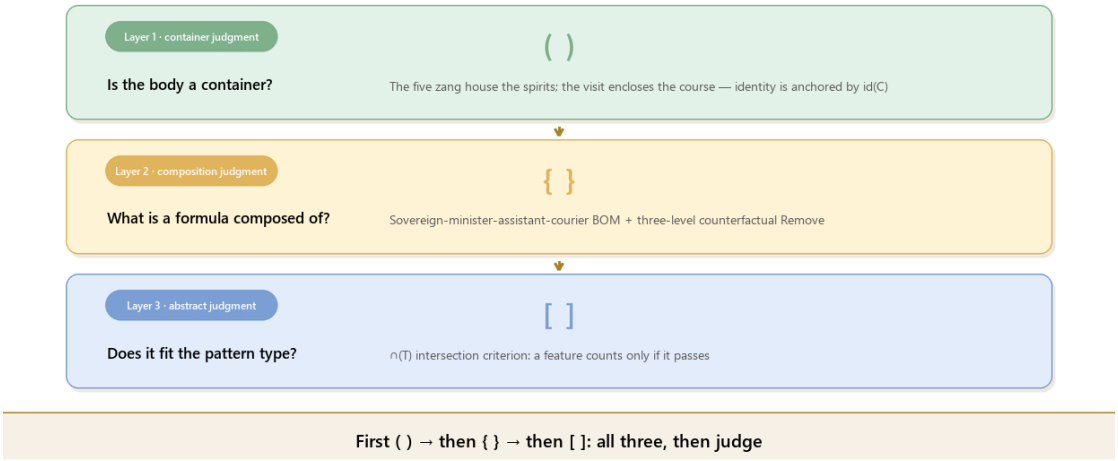


Figure 4-2 Three-layer judgment: body, formula, pattern

The order of the three layers cannot be scrambled: confirm the container and identity first, or you will attach the wrong record (applying someone else's case to this patient); then look at composition, or you will discuss herbs divorced from the formula's structure; only then make the abstract classification, so the criteria have something to attach to. Cases that get stuck usually skip a layer - talking about the pattern type while skipping the container is the commonest form.

4.4 Pick, Combine and Swap Moves

The Time Type's move board is not fully deployed at every visit; moves are picked by situation, combined by need and swapped by result. Pick moves by where the situation is difficult: if the pattern type is hard to differentiate, use "analysis plus reasoning"; if the formula is hard to compose, use "design search and extraction plus design patterns"; if the therapeutic effect will not move, use "problem solving plus restructuring and recombination". Combining moves strings front and back into a pipeline - S1 to S7 is itself a standard combination. Swapping moves is a diagnostic act: if this road will not move, go back one link, swap a move and push again.

Table 4-3 Move selection sheet (situation → moves)

Situation	preferred move	Combined usage
Four-diagnostic information in disorder	Analysis · Type time-function search	Analyse and split the fields first, then fill missed questions from similar cases
Pattern type hard to differentiate, signs atypical	Reasoning · thinking methods	Cross-validation: forward inference from signs to pattern plus reverse testing of the pattern from the formula
Formulas hard to compose, modification hesitant	Design search and extraction · design patterns	First search similar cases for established formula patterns, then adjust according to the BOM
Poor therapeutic effect; the cause must be found	Problem solving · reverse reasoning	Work back from failure: was F(T) wrong? Was the main mechanism misread? Was the dose insufficient?
After several visits it must sharpen	Continuous learning, self-evolution	Case-file review, writing conclusions back into the criteria

Chapter focus

The four-step framework clarifies the need first: fix the object, the scope, the goal and the constraints, then enter the six steps.

The six steps correspond to the output Types and the four layers of accuracy: three preparation steps, one generation step, one acceptance step and one evolution step.

The three-layer judgment runs from outside inward: (container) → {composition} → [abstract]; skipping a layer is the commonest cause of getting stuck.

Moves must be picked, combined and swapped; S1-S7 is itself a standard combination, drilled immediately in Chapter 5.

Chapter Five The S1-S7 Prompt Pipeline and the Insomnia Case



This chapter is the drill ground. The seven prompts S1-S7 correspond to the six output Types plus the validation report, and each is written as "task → input Type → output Type → criteria → prohibitions": the previous station's output is the next station's input, Type locked into Type. All of it is run through Ms. Lin's insomnia case - copy the prompts, swap in your own patient, and you have a usable method.

Figure 5-1 S1-S7 prompt pipeline

Seven prompts, one Type each, each output is the next input

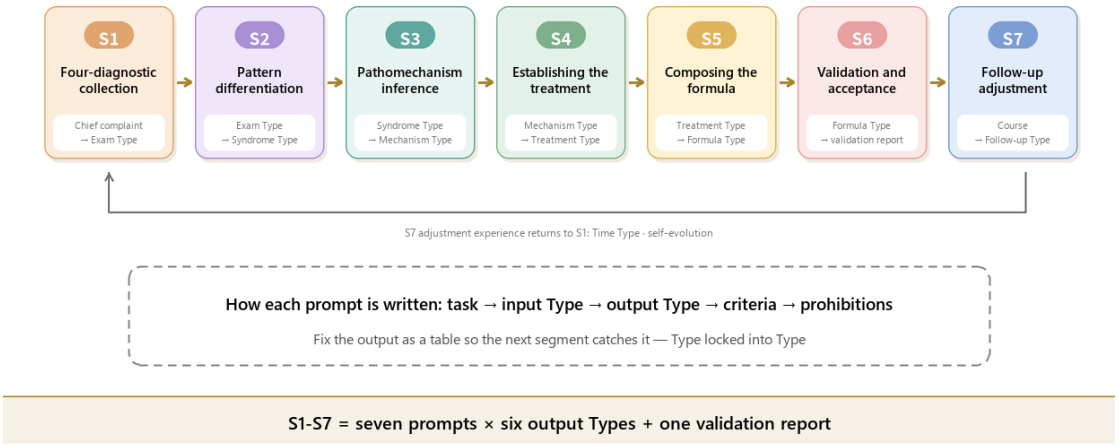


Figure 5-1 The S1-S7 prompt pipeline

5.1 S1-S7 at a Glance

Start with the whole picture: each of the seven stages handles one stretch of the road, and every input and output is an explicit Type. If validation at any stage sends the work back, return to that stage and redo it - the return paths are written in the last column of the table.

Table 5-1 Master table of S1-S7

Stage	Task	Input Type	Output Type	Fail and return
S1	Four-diagnostic collection	Chief complaint dialogue	Exam Type	Missing field → follow-up question
S2	Pattern differentiation	Exam Type	Syndrome Type	∩(T) fails → return to S1
S3	Pathomechanism inference	Syndrome Type	Mechanism Type	Broken link → return to S2
S4	Establishing the treatment method	Mechanism Type	Treatment Type	No correspondence → return to S3
S5	Composing the formula	Treatment Type	Formula Type	Validation fails → return to S5
S6	Formula validation	The complete S1-S5 volume	review report	Any layer fails → return that section
S7	Follow-up adjustment	Previous case file plus the new four diagnostics	Follow-up Type	Therapeutic effect doubtful → reopen the file

Figure 5-2 Insomnia case: one complete consultation and prescription

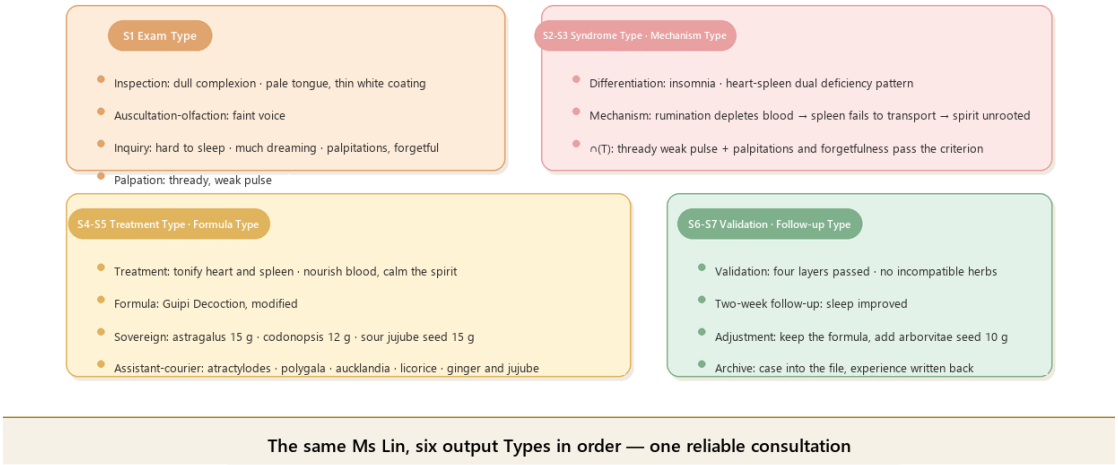


Figure 5-2 The insomnia case: one complete consultation and prescription

5.2 S1 Prompt: Four-Diagnostic Collection

S1 has exactly one task: turn the dialogue into a structured four-diagnostic record. The commonest mistake is slipping into judgment - which is why the prompt explicitly forbids writing a pattern type at this step.

Prompt S1: four-diagnostic collection

```
[Task] You are a TCM consultation assistant. Organise the patient's chief complaint and
dialogue into a structured four-diagnostic record (Exam Type).
[Input] The patient's chief complaint and dialogue record.
[Output format] A four-column table: inspection, auscultation-olfaction, inquiry,
palpation; each column lists the concrete findings.
[Criteria] Tongue and pulse findings are mandatory; symptoms must include degree and
duration; missing fields are marked "not collected" and listed in a follow-up question
```

```
list.
[Prohibited] Do not write a pattern type or a prescription at this step; do not fill
uncollected fields with speculation.
```

Table 5-2 S1 output example: Ms. Lin's Exam Type

Field	Output content
Inspection	Pale complexion, rather thin build, pale tongue, thin white coating, listless spirit
Auscultation-olfaction	Faint voice, no special odour
Inquiry	Insomnia for about two months: difficulty falling asleep, much dreaming and easy waking, waking two to three times a night; palpitations and forgetfulness; poor appetite and dull intake; fatigue and weakness; much brooding; scanty pale menses; normal bowels and urine
Palpation	Thin-weak pulse, abdomen soft without tenderness
Not collected	(None in this case; if present, list it on the follow-up question list, e.g. "recent medication history to be supplied")

5.3 S2 Pattern Differentiation and S3 Pathomechanism Inference

S2 converges the Exam Type into a Syndrome Type; S3 expands the Syndrome Type into a Mechanism Type. One closes, one opens: closing must pass ∩(T), opening must be evidenced link by link.

Prompt S2: pattern differentiation

```
[Task] Organise the Exam Type into a Syndrome Type.
[Steps] (1) Locate by the eight principles (2) Locate by zang-fu organs (3) List the
feature set F(T) (4) Compare candidate pattern types using the intersection criterion
∩(T) (5) Write out the exclusion differential.
[Output] Disease name and pattern type name + the F(T) list + the ∩(T) comparison + an
exclusion statement (why at least one similar pattern type is not chosen).
[Prohibited] When features are insufficient, output "pattern type undetermined" and
list a follow-up question list; never guess.
```

Prompt S3: pathomechanism inference

```
[Task] Expand the Syndrome Type into a Mechanism Type: a causal chain of cause →
mechanism → pattern.
[Output] A three-column chain table: link | content | four-diagnostic evidence; every
link must point back to what the four diagnostics found.
[Prohibited] Do not introduce aetiological assumptions absent from the four
diagnostics; any link that cannot point back to evidence must be deleted.
```

Table 5-3 S2-S3 output example: Syndrome Type and Mechanism Type

Output	Content
S2 · Syndrome Type	Insomnia · heart-spleen dual deficiency pattern
S2 · F(T)	Difficulty falling asleep with much dreaming and easy waking, palpitations and forgetfulness, poor appetite and dull intake, fatigue and weakness, pale complexion, pale tongue with thin white coating, thin-weak pulse, scanty pale menses
S2 · ∩(T)	Thin-weak pulse + pale complexion + palpitations and forgetfulness pass the "heart-spleen dual deficiency" criterion; among the eight principles this is deficiency, and by zang-fu localisation it is heart and spleen

Output	Content
S2 · exclusion differential	Not liver depression transforming into fire (no irritability, bitter taste or wiry-rapid pulse); not yin deficiency with hyperactive fire (no heat in the five centres, no red tongue with little coating)
S3 · cause	Excessive brooding (inquiry: much brooding, clerical work)
S3 · mechanism	Stealthily consuming heart blood → the spleen fails to transport → qi and blood have no source → the spirit is deprived of nourishment (evidence: palpitations and forgetfulness, poor appetite and dull intake, fatigue)
S3 · pattern	Heart-spleen dual deficiency causing insomnia (evidence: pale complexion, pale tongue, thin-weak pulse)

5.4 S4 Establishing the Treatment and S5 Composing the Formula

S4 builds the bridge, S5 crosses it. S4's output is the Treatment Type: one pathomechanism, one method. S5's output is a complete prescription sheet - base formula, sovereign-minister-assistant-courier, modifications, decoction and administration, and physician's instructions, with no column missing.

Prompt S4: establishing the treatment method

```
[Task] From the Mechanism Type, establish the Treatment Type.
[Output] The therapeutic principle (e.g. tonify what is deficient) + the treatment method (e.g. tonify and benefit the heart and spleen, nourish the blood and calm the spirit) + its classification among the eight methods + one sentence on "why this method".
[Criteria] Each link of the treatment method must correspond to the pathomechanism chain; one pathomechanism, one method, no more and no less.
```

Prompt S5: composing the formula

```
[Task] From the Treatment Type, produce a Formula Type (prescription sheet).
[Output format] (1) The base formula name (2) Four columns for sovereign, minister, assistant and courier (herb + dose + task) (3) Modifications (each with its reason noted) (4) Decoction and administration plus physician's instructions.
[Criteria] Every herb must point back to the treatment method; doses must be within the customary range; check combination contraindications (the eighteen incompatibilities and nineteen mutual antagonisms) and special-population warnings first.
[Statement] Mark at the end: "For demonstration only; in clinical practice diagnosis and prescription are made by a qualified TCM physician."
```

The S5 output example is the prescription sheet below. Read it against the BOM in section 2.5: sovereign-minister-assistant-courier is the structural column, modifications are the dynamic column, and the instructions are the advisory column - only with all three does the Formula Type land.

Figure 5-3 Formula-basket output example (S5)

The full landing format of Formula Type

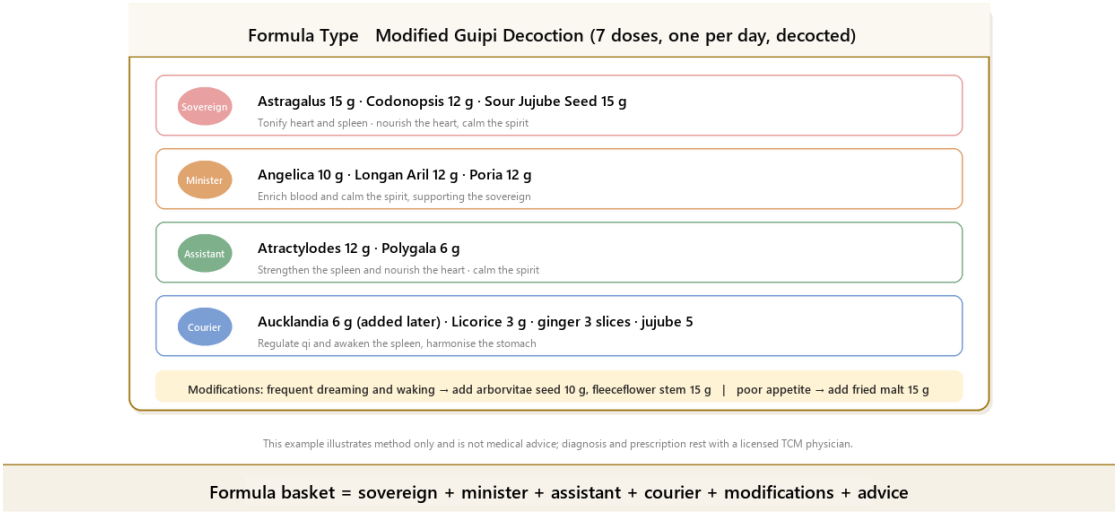


Figure 5-3 A prescription sheet output example (S5)

Table 5-4 S5 output example: prescription sheet (Guipi Decoction with modifications, for demonstration only)

Column	Content
Base formula	Guipi Decoction with modifications, seven doses, one dose daily decocted in water
Sovereign	Astragalus root 15 g, codonopsis root 12 g, sour jujube seed 15 g - tonify and benefit the heart and spleen, nourish the heart and calm the spirit
Minister	Angelica root 10 g, longan aril 12 g, Poria with hostwood 12 g - nourish the blood and calm the spirit, assisting the sovereign
Assistant	Atractylodes rhizome 12 g, polygala root 6 g - strengthen the spleen and assist transport, calm the spirit and open the orifices
Courier	Aucklandia root 6 g (added near the end), licorice root 3 g, fresh ginger 3 slices, jujube 5 pieces - regulate qi and awaken the spleen, harmonise the herbs
Modification	Much dreaming and easy waking, marked → add Biota seed 10 g and Fleeceflower stem 15 g (strengthen the spirit-calming task); poor appetite and dull intake → add stir-fried malt 15 g (assist transport and open the appetite)
Decoction and administration	One dose daily, decocted in water and taken warm in two divided doses, the evening dose warm at bedtime
Physician's instructions	Avoid strong tea and coffee; keep regular bedtimes; soak the feet in warm water before sleep; follow up in two weeks

Example statement: the herb doses in Table 5-4 serve only to demonstrate the method and do not constitute medical advice; in clinical practice please have a qualified TCM physician diagnose and prescribe, adjusted to individual constitution and regulation.

5.5 S6 Formula Validation and S7 Follow-up Adjustment

S6 is the last gate before the formula leaves: run the four layers of accuracy one by one and produce a validation report. If any layer fails, name the break point and send that section back for rework - a return is not a failure, it is this pipeline protecting itself.

Prompt S6: formula validation

```
[Task] A four-layer accuracy formula validation report.
[Input] All outputs of S1-S5.
[Output] A four-layer checklist: (1) ∩(T) cross-check (2) link-by-link inspection of
the pathomechanism chain (3) check of sovereign-minister-assistant-courier and
combination contraindications (4) evidence-chain backtracking. Each layer is given
"pass / fail plus reason".
[Handling] If any layer fails: name the section to be returned (S1-S5); never release a
formula carrying a doubt.
```

Figure 5-4 S6 formula validation: the final gate before prescribing



Figure 5-4 S6 formula validation: the last gate before the formula leaves

Table 5-5 S6 validation report example

Layer	Check	Result
One	Cross-check F(T) against ∩(T)	Pass: the eight principles, the zang-fu localisation and the feature set all match
Two	The pathomechanism chain link by link (cause, mechanism, pattern, method, formula, herb)	Pass: all six links are supported by four-diagnostic evidence
Three	Sovereign-minister-assistant-courier plus the eighteen incompatibilities and nineteen mutual antagonisms plus doses	Pass: structure complete, no combination contraindication, doses within the customary range
Four	Evidence-chain backtracking	Pass: every herb points back to the treatment method, every clause of the method points back to the pathomechanism

S7 reconnects time: at the two-week follow-up, bring the previous case file and compare, decide whether to keep, modify or change the formula, and write this round's conclusions back into the case file. This is the closing move of the Time Type - therapeutic effect becomes experience, and experience becomes criteria.

Prompt S7: follow-up adjustment

```
[Task] Adjust the formula at follow-up and archive it.
[Input] The previous case file (threaded by id(C)) plus this visit's four diagnostics.
[Output] (1) Therapeutic-effect comparison (the difference between the before and after four diagnostics) (2) Whether the pattern type changed (3) The adjustment decision (keep, modify or change the formula) and its reason (4) An evolution record (what was learned this time).
[Criteria] The reason for adjustment must point back to the therapeutic-effect comparison; changing the formula requires re-running S2-S6.
```

Table 5-6 S7 follow-up adjustment example (two weeks later)

Column	Content
Therapeutic-effect comparison	Able to fall asleep (latency shortened); night awakenings down from two to three times to once; palpitations reduced; appetite slightly returning; tongue and pulse unchanged
Pattern-type change	None: still heart-spleen dual deficiency, the direction of F(T) reinforcement is correct
Adjustment decision	Keep the formula and modify: add Biota seed 10 g to strengthen spirit-calming (dreaming persists); seven more doses
Evolution record	This case confirms that the "heart-spleen dual deficiency → Guipi Decoction" criterion works; "appetite not fully recovered" suggests the spleen can be attended to again next visit - write back into the case file

Chapter focus

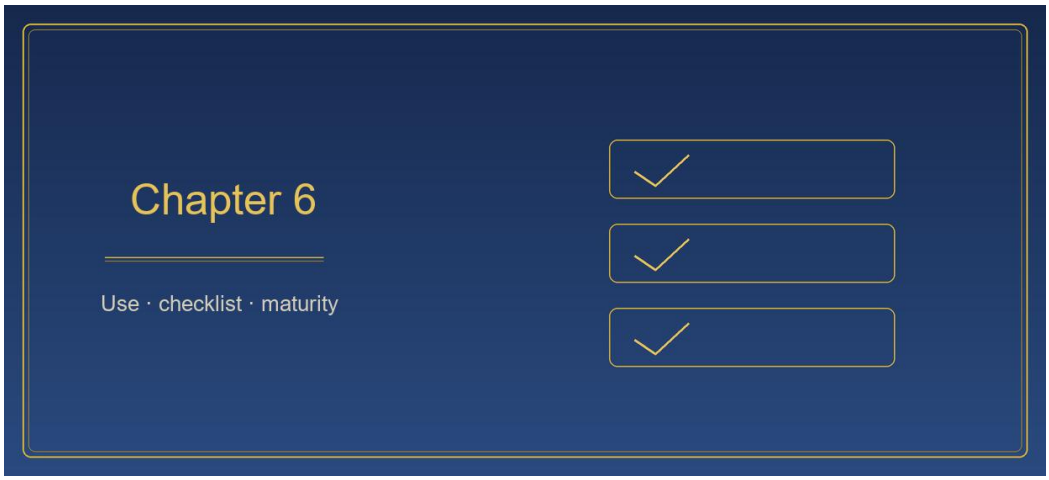
S1-S7 = seven prompts × six output Types + a validation report; each prompt follows a fixed shape: task → input Type → output Type → criteria → prohibitions.

S1 collects only, never judges; S2 converges through ∩(T); S3 expands with evidence; S4 one pathomechanism, one method; S5 a complete prescription sheet.

If S6 fails any of the four layers the work is returned; S7 compares at follow-up, adjusts the formula and archives - experience is written back, and prescriptions get more accurate over time.

Formula and herb content is for demonstration only; in clinical practice a qualified TCM physician diagnoses and prescribes.

Chapter Six Misuse, Checklists and Maturity



The final chapter closes with three pieces of protective gear: the five kinds of misuse keep you out of the commonest pits, the ten-question checklist lets you self-check before a formula leaves, and maturity L1-L4 tells you which level you are at and where to grow.

Figure 6-1 Five misuses: what is wrong, what the right answer is

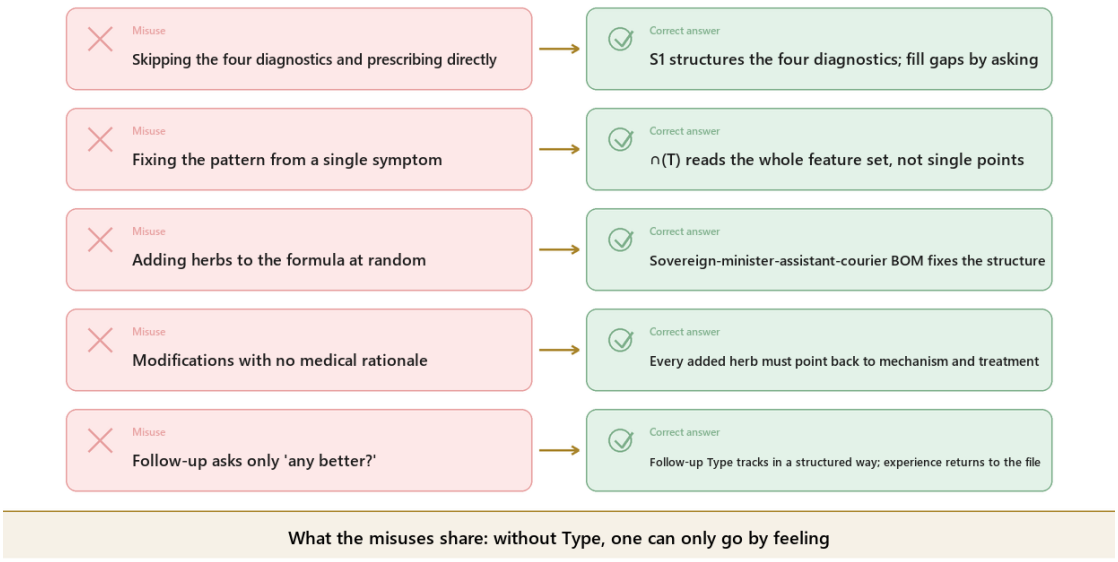


Figure 6-1 Five kinds of misuse: where the error is, and the correct answer

6.1 Five Misuses

The five kinds of misuse share one trait: without the Type, all that is left is feeling. The first four happen inside the process; the fifth happens outside it - treating follow-up as a mere courtesy call.

Table 6-1 Comparison of the five kinds of misuse

#	Misuse	Why it is wrong	Correct answer
1	Skipping the four diagnostics and prescribing directly	Without the Exam Type the evidence chain breaks at its first step	S1 structures the four diagnostics first; missing fields trigger follow-up questions

#	Misuse	Why it is wrong	Correct answer
2	Setting a pattern type from a single symptom	∩(T) looks at the whole feature set, not a single point	Collect F(T) fully, then compare, then write the exclusion differential
3	Assembling herbs at random into a formula	No BOM structure; herbs whose task cannot be stated	Sovereign-minister-assistant-courier sets the structure; the remove counterfactual self-checks it
4	Modification without medical rationale	Add one herb that points nowhere in the pathomechanism and the formula falls apart	Each added herb carries its reason, pointing back to the treatment method
5	At follow-up only asking "feeling better?"	If therapeutic effect is not structured, experience cannot settle	The Follow-up Type's four columns are filled as usual and written back into the case file

6.2 A Ten-Question Checklist

Before the formula leaves, pass ten questions one by one. If any question cannot be answered, return to the corresponding step - this checklist is the pocket edition of the four-layer accuracy guarantee.

Table 6-2 The ten-question checklist

#	Problem	Corresponding layer
1	Is the Exam Type complete? Do tongue and pulse have fields, and are gaps marked?	Evidence chain
2	Is F(T) complete? Are degree and duration written clearly?	∩(T)
3	Has the ∩(T) comparison been run? Can the exclusion differential be written out?	∩(T)
4	Does every link of the pathomechanism chain point back to the four diagnostics?	Deductive skeleton
5	Does the treatment method correspond one-pathomechanism-one-method to the pathomechanism?	Deductive skeleton
6	Can every herb's task be stated and pointed back to the treatment method?	prescription review
7	Is sovereign-minister-assistant-courier complete? Is there a herb whose task cannot be stated?	prescription review
8	Have the eighteen incompatibilities and nineteen mutual antagonisms been checked? Are doses within the customary range?	prescription review
9	Have special-population warnings been checked (pregnancy, menstruation, allergy)?	prescription review
10	Backtracking from the prescription to the four diagnostics, can every step answer "on what grounds"?	Evidence chain

6.3 Maturity from L1 to L4

Maturity has four levels; what is measured is not how large the model is but how thick your case file is and how refined your criteria are. L1 can ask: four-diagnostic collection is complete, no field missing. L2 can differentiate: differentiation has criteria, and the reasons for ∩(T) and the exclusion differential can be stated. L3 can prescribe: the formula comes from the method, all four validation layers pass, and modifications are justified. L4 can evolve: cases are archived and written back, cross-acceptance is done honestly, and prescriptions get more accurate with use.

Figure 6-2 Maturity from L1 to L4: one step at a time

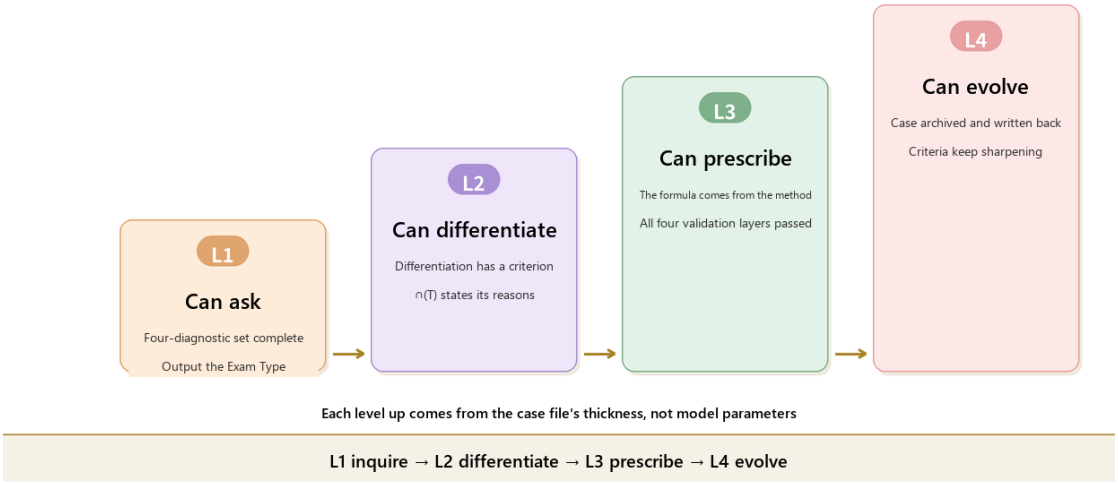


Figure 6-2 Maturity from L1 to L4: one step at a time

Table 6-3 Maturity L1–L4

Level	Name	Target metric	Upgrade action
L1	Can ask	Exam Type complete; not-collected items marked	Turn the follow-up question list into a habit
L2	Can differentiate	∩(T) comparison plus exclusion differential can be produced	Keep an F(T) record at every differentiation
L3	Can prescribe	All four validation layers pass; modifications have reasons	The validation report enters the file, and the return rate falls
L4	Can evolve	Cross-acceptance becomes routine; criteria keep sharpening	Case-file review + move restructuring

> **Chapter focus**
>
> The five kinds of misuse share a common trait: the Type is missing; the ten-question checklist is the pocket edition of the four-layer accuracy guarantee, passed question by question before a formula leaves.
>
> Maturity is measured by the thickness of the case file: L1 can ask → L2 can differentiate → L3 can prescribe → L4 can evolve.
>
> Returns and reviews are not setbacks; they are the only path to sharper criteria.

Review and close

Fold the whole book into one figure. The six output Types are the things, the four layers of accuracy are the gates, the six steps are the road, and S1-S7 is the feel of each move; the Space Type sees "person, pattern, formula" clearly, the Time Type carries "diagnose, treat, adjust" through, and Mind AI bears the whole journey.

Figure R-1 One map for the whole book

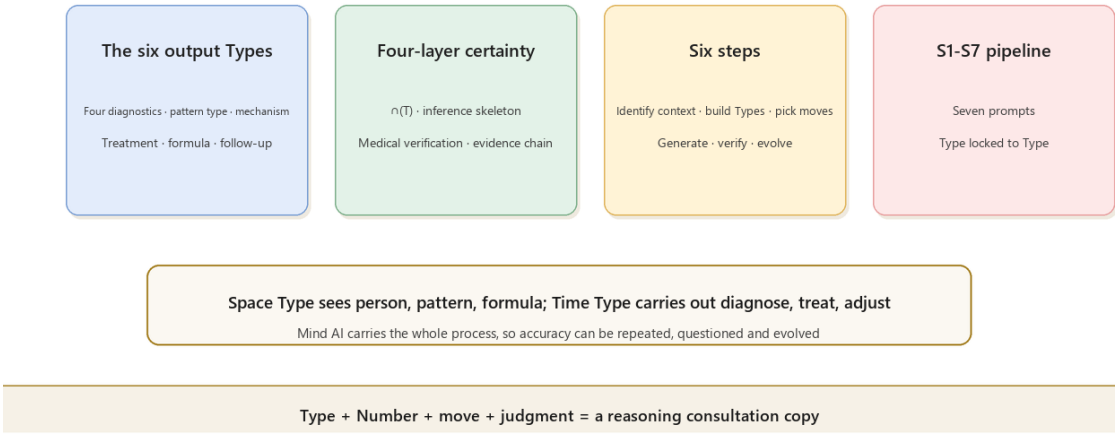


Figure R-1 The whole book in one figure

Take away three things. First, accuracy is structural: the output is a Type, the criterion is ∩(T), the evidence is a chain - with all three, one may speak of accuracy. Second, the method is repeatable: follow S1 to S7 and every visit runs on the same road, with return paths and evolution records. Third, getting more accurate with use is real: follow-ups write back into the case file, cross-acceptance sharpens the criteria, and L4 is not the end but the beginning.

Table R-1 Book-wide checklist

Chapter	What to take away	self-check
Chapter 1 (orientation)	A reason-able replica; Type plus Number; where the two Types land	Can explain this method to a colleague in one sentence
Chapter 2 (output)	The six output Types and the three brackets	Can state the fields and criteria of the six Types
Chapter 3 (accuracy)	four gates of accuracy	"Why is it accurate?" has four answers
Chapter 4 (operation)	Four-step framework + six steps + three-layer judgment	Follow the steps and one visit does not get stuck
Chapter 5 (drill)	S1-S7 and the insomnia case	Swap in your own patient and the pipeline still runs
Chapter 6 (advanced)	Misuse, ten questions, maturity	Know which L you are at and what to practise next

Appendix A Quick Reference: Terms, Moves, Notation

The terms, moves and notations used in this book are gathered in three tables. For theoretical detail return to the original Ontypology texts; here the only requirement is that they can be looked up when in use.

Table A-1 Terminology quick reference

Terminology	English / notation	In this book it means
Type	Type	A structured output unit; in English always Type, never Form
Exam Type	ExamType	A structured record of inspection, auscultation-olfaction, inquiry and palpation
Syndrome Type	SyndromeType	Differentiation conclusion: disease name + pattern type, F(T) passing ∩(T)
Mechanism Type	MechanismType	Cause→mechanism→pattern causal chain
Treatment Type	TreatmentType	Therapeutic principle + treatment method + classification among the eight methods
Formula Type	FormulaType	Sovereign-minister-assistant-courier BOM plus modifications plus instructions
Follow-up Type	FollowType	Therapeutic-effect comparison + adjustment decision + evolution record
Feature set	F(T)	The total set of features the Syndrome Type admits
Intersection criterion	∩(T)	The rule that decides "does it belong to this Type"
Identity anchor	id(C)	Identity anchoring of the container (patient, course of illness)
Composition Set	{ }	BOM: what it is composed of
Container Set	( )	Where does it live, and who is it
Abstract Set	[ ]	Which class it falls into

Table A-2 The Time Type's fourteen moves × Chinese medicine

Set	Move	Where consultation and prescribing lands
Find moves	Type-time function search	Retrieve similar cases using the Exam Type
Find moves	Design search and extraction	Extract pattern-type features and established formula patterns from cases
Find moves	Design patterns	Apply a combination pattern (e.g. the sovereign-minister-assistant-courier structure)
Make the move	Logic generation and application	Reasoning generation for pattern differentiation and treatment
Make the move	Function generation and application	Generation of the treatment method and the formula
Make the move	Design (creation and innovation)	Formula composition and modification - prescribing is design
Devise moves	Problem solving	Attribution and countermeasures for poor therapeutic effect
Devise moves	Analysis	Splitting four-diagnostic information into fields, pattern-type differentiation
Devise moves	Reasoning (causal, reverse-causal, simulation, statistical, reverse)	The pathomechanism causal chain; reverse verification of the pattern from the formula
Devise moves	Thinking methods (forward, reverse, horizontal, vertical, first principles)	Multi-angle differentiation of difficult cases

Set	Move	Where consultation and prescribing lands
Practise the moves	Continuous learning	Case archiving, experience accumulation
Practise the moves	Refactoring and recombination	Adjustment restructuring, formula regrouping
Practise the moves	R&D and exploration	New-formula exploration, treatment-method probing
Practise the moves	Self-evolution	Cross-acceptance writes back into the criteria, so prescriptions get more accurate

Table A-3 Notation quick reference

Notation	Reading path	purpose
[ ]	Square brackets	Abstract set: does it belong to [insomnia]
( )	Parentheses	Container sets: (Ms. Lin), (this visit)
{ }	Braces	Composition set: the BOM of {Guipi Decoction with modifications}
∩(T)	Intersection criterion	The membership rule of a Type
F(T)	Feature set	The total set of features the Syndrome Type admits
id(C)	Identity anchor	Identity anchoring of the container
remove	removal criterion	Three-level counterfactual: single herb / herb pair / formula root

Book-wide statement: the formulas and herbs in this book (including the herbs, doses and modifications in Ms. Lin's case) serve only to demonstrate the method and do not constitute medical advice; for clinical care please have a qualified TCM physician diagnose and prescribe.

Type is Type, not Form. Theory is used, not taught; the platform is cited, not described.

MIND AI

The eternal path of AGI

Unified knowledge · unified thinking · unified intelligence

Ontypology · Mind Model · Mind AI

Space Type (Type) + Time Type (move) = reasoning copy



AI TCM consultation and prescribing: full output and usage plan

Formula and dosage are for method illustration only; not medical advice